

What the Research Says: Donors, Caregivers, and the Gaps Nobody Measures

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Every week, hundreds of papers about kidney disease and transplantation appear in journals most families will never open. Most are written for other researchers. A few contain something a patient or care partner could use tonight. This is the first of our weekly reviews: four 2026 studies, what each found, what it cannot tell us, and the question each one hands you to take to your care team. We have no donors and no industry funding, so we choose by a single test: does this matter to the person living it? Each item below also carries the line the big donor-funded organizations rarely print: who paid for the work, or a plain statement that we have not yet been able to confirm it.

Living donors: the long studies say kidney function holds

Anyone considering giving a kidney is usually offered one reassuring sentence about the long term. What does the evidence actually say? A Swiss registry followed 1,863 living kidney donors for at least five years, from donations made between 1993 and 2018. Their kidney function stayed stable, with an average yearly change of +0.14 in eGFR, the standard measure of how well the kidneys filter. A Heidelberg cohort followed 632 donors from 1991 to 2020 and saw the same pattern after the early, expected drop, whether donors were under forty, between forty and sixty, or over sixty.

1,863

LIVING KIDNEY DONORS FOLLOWED FIVE YEARS OR MORE IN ONE
SWISS REGISTRY

Not every study agrees. A small Turkish comparison of 87 donors and 100 matched controls found that nephrectomy was an independent risk factor for high blood pressure and for a lasting drop in filtering function of more than twenty percent. The honest reading is that the two large studies are reassuring for most donors, that the older-donor result matters for people considering donation later in life, and that a small single-center study is a reason to ask rather than to panic. These are observational studies, which can show association but not cause, and donors who stay in follow-up may be healthier than those who disappear from it. The practical question for a program is simple: what are my numbers, not the average's? Ask what happens to donors who develop high blood pressure, who pays for the yearly checks, and how the program tracks donors after the first two years. Those answers tell you more about a program than a reassuring average ever will.

The caregiver at the kitchen table

Almost all transplant research counts the patient. A 2026 interview study counted the person beside them. Twenty-three caregivers, eleven spouses, ten parents and two siblings, described what it takes to support someone through the loss of a transplanted kidney: managing diet, supporting dialysis at home, navigating a fragmented system, and holding every other role in the family at the same time. They reported feeling undervalued, and the authors recommend that caregivers be better included, supported and educated.

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CAREGIVERS DESCRIBED SUPPORTING A RECIPIENT THROUGH GRAFT
LOSS

This is a small qualitative study, so it cannot say how common these experiences are. What it can do is name something the system rarely measures. If you are a care partner, the exhaustion this study describes is not a personal failing. It is a documented feature of the work, and it is fair to ask the transplant team what support exists for you, not only for the patient. Ask for a social worker, a caregiver education session, and a name to call when the plan changes. Programs are paid to treat patients; caregivers have to ask to be counted.

When a transplant fails, what blocks the next one

A transplanted kidney does not last forever, and many families face a second search for a donor. A study published in the Clinical Journal of the American Society of Nephrology in July 2026 asked patients, caregivers and providers what stands in the way of retransplantation after graft loss. They named barriers that are medical and barriers that are not, along with additional obstacles specific to pursuing a second living-donor transplant.

2026

CJASN STUDY OF BARRIERS TO RETRANSPLANTATION AFTER GRAFT LOSS

The study records perceptions, not how often each barrier occurs, so it should not be read as a ranking. Its value is that a named barrier is one you can raise early. If your transplant is failing, or has failed, ask your team which of these barriers apply to you and who can help with each one, before the clock on your options gets short.

The rule changed. Most patients were still left out.

In 2022 the transplant system stopped using a race-based adjustment in the kidney-function formula, an adjustment that had kept many Black patients lower on the waiting list. Eligible candidates were supposed to have their waiting time restored. A single-center study found 274 Black patients eligible, and only 109 of them, 39.8 percent, actually received the modification. Those who did received an average of 2.7 additional years of wait time. The main obstacle was finding old laboratory results to prove when kidney function had first fallen low enough.

109 of 274

BLACK PATIENTS WHO RECEIVED RESTORED WAIT TIME AT ONE LARGE CENTER

It is one center, not the nation, and national analyses report different totals that should be read alongside it. But the lesson travels. A policy can be correct on paper and still miss people at the bedside. If you are a transplant candidate, ask your program whether your waiting time was reviewed under the race-neutral rule, and help find your old labs if they are needed.

What to take from this week

Four studies, one thread: the research is strongest on the patient, thinner on the people around the patient, and almost silent on whether good policy actually reaches the people it was written for. That is the gap we intend to keep pointing at, every week, with the source in your hands so you never have to take our word for it. If you read one of these papers and find we got something wrong, tell us and we will correct it in public. Independence is not a claim we make about ourselves; it is a standard you are invited to check, number by number.

Sources

- Heidelberg living-donor cohort (632 donors, 1991–2020). Transplant International, 2026. DOI 10.3389/ti.2026.16266.
- Swiss Organ Living-Donor Health Registry (1,863 donors). Kidney International Reports, 2026. Full DOI not confirmed.
- "Long-term clinical outcomes in living kidney donors: a comparative study with matched controls." PMID 41175287. DOI 10.1007/s11255-025-04880-w. Publication year not confirmed.
- "Caregiver Perspectives on Supporting Kidney Transplant Recipients Through Graft Loss." 2026. PMID 42088030.
- "A Qualitative Study of Patients, Caregivers, and Providers on Barriers to Kidney Retransplantation following Graft Loss." Clinical Journal of the American Society of Nephrology, 2026. Full DOI not confirmed.
- "A Single-Center Assessment of the Race Neutral eGFR Calculation and Access to Kidney Transplantation for Black Patients: A Policy Change Is Not Enough." Journal of Racial and Ethnic Health Disparities. DOI 10.1007/s40615-025-02573-9. PMID 41811668.

How we checked

We could not open the journal websites while preparing this issue, so every figure comes from the study titles, abstracts and summaries returned by search and indexing pages, cross-checked where two sources were available. Funding and conflict-of-interest disclosures: not reviewed by CIF; see each source article. We did not confirm the full DOI of the Swiss registry paper or the CJASN article, the exact unit behind the Swiss eGFR figure, or the publication year of the Turkish comparison and the single-center eGFR study, so those details are described as reported in the sources and should be verified there. If you find an error, tell us and we will correct it in public.

Educational, not medical advice. Talk to your healthcare team. In an emergency, call 911.